Clinical Study Summary Report (CSSR)
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1. Administrative & Study Identification

Full Study Title: A Phase 1, Double-blind, Placebo-controlled, Randomized Study of the Safety, Tolerability, and Pharmacokinetics of BMS-986419 (Part 1) and a Phase 1 Randomized, Double-blind, Positive-controlled, Placebo-controlled, Parallel, Nested-crossover (Moxifloxacin-placebo) Thorough QT/QTc Study to Evaluate the Effect of Multiple Doses of BMS-986419 on Cardiac Repolarization (Part 2) in Healthy Participants **Short Title/ Acronym:** A Study to Investigate the Safety, Tolerability, and Drug Levels of BMS-986419 (Part 1) and the Effects Multiple Doses of BMS-986419 on Cardiac Repolarization (Part 2) in Healthy Participants **Protocol Number:** CN007-1005 **NCT Number:** NCT06846866 **Sponsor Name:** Bristol Myers Squibb **Investigational Product:** BMS-986419 (also known as EVT-8683 or Comtifator; an eIF2B activator) **Phase of Development:** Phase 1 **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

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2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To investigate the safety, tolerability, and pharmacokinetics of BMS-986419 (Part 1) and to evaluate the effects of multiple doses of BMS-986419 on cardiac repolarization (Part 2) in healthy participants. **Secondary Objectives:** To assess specific pharmacokinetic (PK) profile parameters (such as Cmax and AUC) across multiple doses and detailed ECG characteristics.

2.2 Background & Rationale BMS-986419 (Comtifator/EVT-8683) is a small molecule eukaryotic translation initiation factor 2B (eIF2B) activator designed to modulate the integrated stress response (ISR) pathway. It is under development as a potential disease-modifying treatment for neurodegenerative conditions, including Alzheimer's disease and amyotrophic lateral sclerosis (ALS). Before advancing to patient populations, this Phase 1 study is strictly required to establish the human safety profile, tolerability, drug levels, and crucially, any potential for cardiac repolarization (QT/QTc interval prolongation) using a rigorous Moxifloxacin positive-controlled design.

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3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled and Positive-controlled (using Moxifloxacin) **Blinding:** Double-blind **Randomization:** Randomized (Parallel, Nested-crossover for Part 2)

3.2 Planned Enrollment & Duration Target N: 74 **Number of Sites:** Clinical pharmacology units in the USA **Estimated Study Start:** 2025 **Estimated Primary Completion:** October 2025 (Status: Completed)

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4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Ages 18 to 55 years old; all genders. 2. Healthy as determined by medical history, physical examination, vital signs, 12-lead ECG, and clinical laboratory assessments. 3. Body mass index (BMI) between 18.0 and 30.0 kg/m^2, inclusive, at screening.

4.2 Exclusion Criteria 1. Documented diagnosis of Gilbert Syndrome. 2. History of clinically relevant cardiac disease, symptomatic or asymptomatic arrhythmias, presyncope or syncopal episodes, or additional risk factors for ventricular arrhythmias (e.g., long QT syndrome, catecholamine polymorphic ventricular tachycardia). 3. Current or recent (within 3 months of study intervention administration) gastrointestinal (GI), liver, or kidney disease that could possibly affect drug absorption, distribution, metabolism, and excretion (e.g., bariatric procedure, cholecystectomy, and any other GI surgery). 4. Any significant acute or chronic medical illness as determined by the investigator.

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5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Multiple ascending doses of BMS-986419; Single dose of Moxifloxacin (positive control; Trade names: Moxeza, Vigamox, Avelox; ATC Codes: J01MA14, S01AE07). **Route of Administration:** Oral (PO) **Duration of Treatment:** Short-term (Phase 1 confinement period)

5.2 Concomitant & Prohibited Therapy Permitted: Standard meals and hydration as dictated by the in-patient Phase 1 protocol. **Prohibited:** Exposure to any investigational drug or placebo (other than BMS-986419 or moxifloxacin) within 4 weeks or 5 half-lives (whichever is longer) prior to Day -1 (Day -2 for Part 2) until follow-up phone call.

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6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :--- | :--- | :--- | | **Screening** | Days -28 to -2 | Informed Consent, Medical History, BMI evaluation, 12-lead ECG, Safety Labs | | **Baseline** | Day -1 / 0 | Admission to clinical unit, Randomization, First Dose | | **Interim** | Treatment Days | Intensive PK blood sampling, continuous/serial ECG monitoring (Thorough QT/QTc assessment), AE monitoring | | **End of Study** |

Follow-up | Final vital signs, safety labs, exit evaluation, follow-up phone call |

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7. Outcome Measures (Endpoints)

7.1 Primary Endpoints Definition: 1. Part 1: Number of Participants with Non-serious Adverse Events (NSAEs) 2. Part 1: Number of Participants with Serious Adverse Events (SAEs) 3. Part 1: Number of Participants with AEs Leading to Discontinuation **Measurement Method:** Continuous safety monitoring, clinical laboratory assessments, and evaluation of cardiac repolarization measured by changes in the QT/QTc interval from baseline after multiple doses (Part 2) read by a central laboratory.

7.2 Secondary & Exploratory Endpoints Definition: 1. Part 1: Maximum Observed Plasma Concentration (Cmax) of BMS-986419 2. Part 1: Time of Maximum Plasma Observed Concentration (Tmax) of BMS-986419 3. Part 1: Area Under the Concentration-time Curve in One Dosing Interval (AUC(TAU)) of BMS-986419

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8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Conducted continuously during in-patient confinement via vital signs, telemetry, physical exams, and structured interviews to capture NSAEs and AEs leading to discontinuation. **Serious Adverse Events (SAEs):** Required standard 24-hour expedited reporting to the sponsor and Institutional Review Board (IRB). **Data Monitoring Committee (DMC):** Internal safety review committee at Bristol Myers Squibb to oversee dose escalations.

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9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all randomized participants receiving at least one dose); PK Evaluable Set (participants with sufficient concentration-time data). **Sample Size Justification:** Sufficiently powered to rule out a clinically significant QT/QTc prolongation (typically an upper bound of the 90% confidence interval > 10 ms). **Handling of Missing Data:** Standard handling for early terminations per Phase 1 thorough-QT guidelines.

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10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** High-frequency, onsite source data verification typical of Phase 1 clinical pharmacology units. **Audit Trail:**

Strict eCRF locking and audit trailing for all cardiac repolarization (ECG) readouts.

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11. Study Identifiers & Governance

Primary ID: CN007-1005 **Posting ID:** 794810858418845835 **Registry Number:** NCT06846866 **Sponsor/Lead Organization:** Bristol Myers Squibb (Company) **Study Title:** A Study to Investigate the Safety, Tolerability, and Drug Levels of BMS-986419 (Part 1) and the Effects Multiple Doses of BMS-986419 on Cardiac Repolarization (Part 2) in Healthy Participants **Official Regulatory Title:** A Phase 1, Double-blind, Placebo-controlled, Randomized Study of the Safety, Tolerability, and Pharmacokinetics of BMS-986419 (Part 1) and a Phase 1 Randomized, Double-blind, Positive-controlled, Placebo-controlled, Parallel, Nested-crossover (Moxifloxacin-placebo) Thorough QT/QTc Study. **Preferred UMLS Name:** Phase 1 Clinical Trials

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12. Clinical Framework (Clinical Pharmacology Focus)

Primary Condition: Healthy Volunteers (Indication target: Alzheimer's Disease / Amyotrophic Lateral Sclerosis; MeSH Code: D064368; SNOMED ID: 106207008) **Diagnosis Threshold:** BMI 18.0 - 30.0 kg/m^2; Normal 12-lead ECG **Medical Methodology:** Integrated Stress Response (ISR) Modulator / eIF2B Activator **Optimization Target:** Drug safety, maximum tolerated dose (MTD), and cardiac safety validation (QTc)

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13. Study Procedures & Methodology

Management Pathway: Intensive Phase 1 in-patient pharmacokinetic and telemetry observation **Education Protocol (HCP):** Investigator brochure training for clinical pharmacology unit staff **Education Protocol (Patient):** Protocol restriction counseling (e.g., prohibition of concomitant meds/food interactions) **Quality Audit Mechanism:** Blinded core laboratory readings for ECGs; verified LC-MS/MS bioanalytical assays for PK

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14. Observation & Follow-Up Schedule

Total Duration: Confined in-patient treatment period plus safety follow-up call **Onsite Visit Cadence:** Daily confinement for the duration of the drug-administration phase **Remote Monitoring Frequency:** N/A (In-patient study) **Checklist Review Interval:** Daily during dosing for AEs and symptom tracking

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15. Sign-off & Approval

Role	Name	Signature	Date
Principal Investigator	[Name]	____	[DD-MMM-YYYY]
Clinical Operations Lead	[Name]	___	[DD-MMM-YYYY]
Lead Statistician	[Name]	_____	[DD-MMM-YYYY]